Evidentra — Secondary Research Report

Prepared 21 Oct 2026 · Desk research only (web sources, no interviews) — **secondary research, not customer validation** · Tags: P primary/government · S secondary reporting/vendor/commercial · A assumption (ours)

1–2. Executive summary and recommendation

Recommendation: REVISE (do not stop; do not build the full product yet). Evidentra targets a real, documented regulatory environment: India licenses nearly all device classes on dossier-style submissions, a parliamentary committee reports repeated query rounds and opacity, and the government is drafting faster statutory timelines. But **no source found measures how often CDSCO queries or delays are caused by missing or inconsistent documents**, no source gives a verified count of Indian licensees or regulatory consultants, and willingness to pay, time saved and accuracy are all unproven. Revise the plan to: (a) lead with **regulatory consultants / small RA firms** (repeat submissions across clients) rather than single-product startups; (b) scope v1 to **CDSCO Class B and C manufacturing licence dossiers**; (c) run the cheap tests on page 3 before any engineering spend. The market model below is an **assumption-driven range (base ≈ ₹63 Cr TAM, ₹19 Cr SAM, ₹1.1 Cr 3-yr SOM)**, not a forecast.

3. Problem validation

Claim	Evidence (date · type)	Confidence	Unknowns
Applicants face repeated clarification rounds and slow, opaque licensing	Parliamentary Standing Committee on Health & FW (reported 13 Mar 2025) criticised "multiple rounds of queries", lack of transparency, and urged a single-query policy and 45-day conditional approvals S medicaldialogues.in. Business Standard coverage could not be opened (HTTP 403); committee report itself not read.	Medium	No counts of queries, pendency or processing times in any source read. Report text unverified.
Regulator and industry see timelines as a problem worth fixing	Draft MDR amendment reported 29 Jun 2026: Class B 140→115 days; Class C/D 105→90 days; stage-wise timelines S biospectrumindia.com. A law-firm summary gives a different Class B figure (95 days) S chambers.com — unresolved; gazette text not checked. Draft, not in force.	Medium	Whether notified; statutory timelines ≠ actual elapsed time, and say nothing about *cause*.
CDSCO dossiers are document-heavy and multi-part	Applications are online (cdscomdonline.gov.in / NSWS); document checklist is set per form on the portal; Plant Master File content is prescribed (Fourth Schedule, Appx I); Essential Principles apply to all classes; QMS per Fifth Schedule P CDSCO FAQ, Doc CDSCO/FAQ/MD/01/2024. Class C/D manufacturing: MD-7/MD-8 → MD-9/MD-10 on SUGAM P CDSCO MD page (page undated; contains 2021 items).	High (that requirements exist)	How much effort/cost applicants spend; which checklist items fail most.
Missing or inconsistent evidence causes rework in comparable regimes (proxy only)	EU: in Team-NB surveys, notified bodies running completeness checks reported ~80% of technical documentation missing about half the needed information (2023) — share of *NB responses*, not of applications S gmp-compliance.org. US: FDA refuse-to-accept ~60% in 2013; "~one-third" recently per blogs S mddionline.com.	Low for India	**No India-specific data found.** Different regimes, formats and gatekeepers; not transferable.
Regulator is open to AI pre-screening	Committee recommended a "time-bound, AI-assisted online licensing process" and AI-based pre-screening S (medicaldialogues.in, above).	Low–Med	Could be opportunity (aligned) or threat (CDSCO builds it in-portal).

4. Indian regulatory landscape and evidence of customer pain

- **Legal base:** Medical Devices Rules, 2017 (G.S.R. 78(E), 31 Jan 2017) under the Drugs & Cosmetics Act; four risk classes A–D P CDSCO MDR page. That page lists 34 notifications; consolidated text dated 15 Feb 2023, latest listed entry 2 Jun 2023 (G.S.R. 409(E)). **Later changes (below) are known only from secondary reports — current consolidated text must be verified before product logic is built.**
- **Scope:** All devices fall under licensing except Class A non-sterile, non-measuring, which are exempt but need registration (G.S.R. 777(E), 14 Oct 2022 P CDSCO MDR page above; 31 Oct 2025 notice S morulaa.com). Class C/D licensing extended to previously non-notified devices from 1 Oct 2023 S ssrana.in.
- **Process change, Oct 2025:** reported CDSCO directive (23 Oct 2025) requires central classification of Class A/B/IVD devices before State authorities grant licences; SUGAM classification module live 27 Nov 2025 (devices) S morulaa.com (above). Class A/B licences: State authority (MD-5) after notified-body audit; C/D: Central authority (MD-9) S altacit.com.
- **Pain evidence (qualitative):** BCG/AiMeD report (May 2026) lists sequential approvals, high certification costs and "longer turnaround for Class C validation due to high dependence on CDSCO" as MSME constraints S BCG PDF. The 2025 committee reported manufacturers shifting to Vietnam/Malaysia (medicaldialogues.in). **Not shown:** that documentation errors, rather than inspection backlog or staffing, drive these delays.
- **Fees (context):** reported govt fees Class A/B ₹5,000/site + ₹500/device; Class C/D ₹50,000/site + ₹1,000/device S agileregulatory.com (sources conflict; verify on CDSCO).

Method and source integrity

Sources were fetched 21 Oct 2026. Primary = CDSCO pages/FAQ read directly; secondary = news, law-firm, consultant and vendor pages (self-interested; unaudited). Not obtained: the committee report itself, Business Standard (HTTP 403), official licensee counts, any CDSCO query/pendency statistics, and the amended consolidated MDR text after Jun 2023. The CDSCO "Medical Device & Diagnostics" page is undated and appears partly out of date. No figure here is invented; unsourced numbers are tagged A. General regulatory delay is not treated as proof of documentation-caused delay.

5. Customer and behavioural evidence

Segment	What the evidence says	Fit for Evidentra (our judgement, unvalidated)
Established manufacturers	Likely have in-house RA/QA; large firms already buy QMS/RIM tools abroad (Rimsys claims 6 of top 12 global MedTech S rimsys.io). India-specific buying behaviour not found.	Low volume of new-India submissions per firm; long enterprise cycles. Later segment.
Startups / MSMEs	BCG: MSMEs face "high certification costs", capital constraints S; CDSCO runs a startup help cell (PRO) P cdsco.gov.in/PRO. ~17% of 150+ incubated MedTech startups expected to reach commercialisation (BCG).	Acute pain, but 1–3 submissions/yr, low budget, may default to a consultant. Poor first payer.
Regulatory consultants / agents	Active market: Morulaa claims 500+ manufacturers served S morulaa.com; Freyr lists Indian authorised-agent services S freyrsolutions.com; Pure Global acts as Authorized Indian Agent S. CDSCO FAQ: only an authorised company person may query application status — consultants file but cannot track on their own P (FAQ above, Q11). Self-reported claims, unaudited.	**Most promising first payer (hypothesis):** repeat dossiers, billable-hour savings, multi-client reuse, faster purchasing. Risk: they may fear disintermediation.

Behavioural gaps: no survey, interview or forum data on how Indian RA teams compile dossiers (Excel/Drive checklists vs software), tool adoption, or consultant fees was found. Treat all behaviour as unobserved until the interviews on page 3.

6. Competitor comparison (vendor pages read 21 Oct 2026; "not stated" ≠ "absent")

Company	Capabilities (as stated by vendor)	Pricing (public)	Target	Potential gap to test
Pure Global	AI-assisted dossier prep; India Authorized Agent; CDSCO registration support; free AI tools (RMF review, change impact)	From $2,000/yr (Class A), $3,000/yr (B–D) per device, excl. govt fees; 3-yr term (pricing)	Foreign & Indian makers needing agent	Service-bundled; evidence-to-clause traceability and cross-document checks not stated
Greenlight Guru	Device-specific eQMS: doc control, change mgmt, AI search/summaries	eQMS "start at $12,000/yr"; implementation extra	Medical-device cos, early to global	CDSCO and submission-mapping not stated
Qualio	eQMS; AI gap analysis; Copilot drafting/review; regulatory monitoring (Growth tier)	Quote only	Life-science cos	CDSCO-specific clause mapping not stated
Rimsys	RIM: submission planning/tracking, 90+ country source monitoring, impact assessments, AI content support	Quote only; "no per-user fees"	Global medtech	India/CDSCO not stated; enterprise price likely high for MSMEs (inference)
RegDesk	RIM; AI forms/submission generator; automated gap analysis; updates from 120 markets	Demo only	Global scaling cos	CDSCO depth unverified; test directly
Cruxi	FDA 510(k) AI workflow, RTA check; other regimes "in beta"	$1,000 per full 510(k) workflow; $500/mo membership (50 credits)	Cos, consultants, agencies	Not India; useful price anchor for per-submission model
MasterControl	Life-science QMS; named-user licences	Quote only	Pharma/med-device	Submission mapping not stated (pricing)
Consultants (Morulaa, Freyr)	Technical files, dossier preparation/review, CDSCO/SUGAM submission	Not stated	Manufacturers	Human review is the incumbent; also potential customers/channel

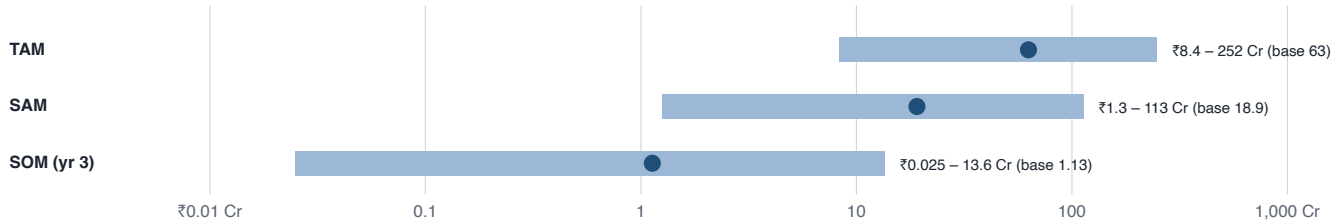
Also unquantified: general-purpose LLM chat tools and spreadsheets (the likely real incumbent). Third-party price estimates (e.g. Greenlight median ~$44k, Capterra listings) were seen but not used: unverifiable.

7. Market evidence and TAM / SAM / SOM

Evidence: India device market ≈ US$15.2 bn FY25 → $50.1 bn FY30, imports $8.6 bn vs exports $4.1 bn S ianslive.in (Rubix); BCG puts it at ~$16–20 bn. The same coverage cites **~800 manufacturers** (secondary, unaudited, not defined as licensees). **No official licensee count or consultant count was found** (Lok Sabha/CDSCO searches returned none) — counts below beyond 800 are A assumptions. ACVs are set directly in ₹; anchors at an assumed ₹88/US$: Cruxi $1,000/submission ≈ ₹0.9 L; Pure Global $2–3k/device-yr ≈ ₹1.8–2.6 L.

Layer / formula	Low	Base	High	Assumptions A
Firms: mfrs N_m / consultants N_c	800 / 300	3,000 / 600	6,000 / 1,200	Low = reported ~800; base/high guess that licensing of all B–D devices widens the base. Consultants unknown.
ACV (₹/yr): mfr / consultant	0.6 L / 1.2 L	1.5 L / 3 L	3 L / 6 L	Willingness to pay unvalidated. 1 L = ₹1 lakh.
TAM = $N_m \cdot ACV_m + N_c \cdot ACV_c$	₹8.4 Cr	₹63 Cr	₹252 Cr	Base: 3,000×1.5L=45 Cr; 600×3L=18 Cr.
SAM = TAM × s	15% → ₹1.26 Cr	30% → ₹18.9 Cr	45% → ₹113 Cr	s = Class B–D, digital, English dossiers, ≥1 filing/yr, SaaS-willing.
SOM (3 yr) = SAM × r	2% → ₹0.025 Cr	6% → ₹1.13 Cr	12% → ₹13.6 Cr	r = share won by a 1–2 person GTM. Base ≈ 54 mfrs + 11 consultants.

8. Opportunity chart



Log scale, ₹ crore per year. Bar = low→high scenario; dot = base. Spread is ~30× (TAM) to ~500× (SOM): the uncertainty is the finding. All inputs are assumptions (table above).

9. Business model and pricing hypotheses (all ₹ figures A, untested)

Model	Hypothesis	Pro	Con / evidence
Per-submission pack	₹15–40k per dossier check (Class B–C); one-off	Matches how startups buy; anchored by Cruxi $1,000/510(k) and consultant project fees; low commitment	Lumpy revenue; 1–3 filings/yr per mfr limits ARPA
Consultant subscription	₹3–6 L/yr, N dossiers + client workspaces	Repeat use; fits multi-client work; closest to Cruxi agency/membership structure	Consultant fee levels unknown; disintermediation worry
Manufacturer subscription	₹0.6–3 L/yr incl. change monitoring	Recurring; ties to post-licence changes and retention	Competes with eQMS suites ($12k+/yr ≈ ₹10 L+) and Pure Global ($2–3k/device-yr)

Lean: sell per-submission packs to consultants first (credits, Cruxi-style), convert heavy users to a seat/client-based plan. Govt fees (≈₹5k–50k+/site) are a reference point for what applicants already spend per filing; whether they cap willingness to pay is unknown.

10. Product differentiation and feasibility

Feature	Differentiation (evidence-based view)	Feasibility / caveat
Requirement → evidence mapping	Competitors advertise gap analysis and forms generation (Qualio, RegDesk) but CDSCO form-level mapping is not stated by any vendor read.	Needs a maintained CDSCO requirement library (checklists live in the portal, per FAQ). Feasible; upkeep is the cost.
Page-level source traceability	Not stated by competitors read; unverified absence. Key trust feature for regulated users.	Technically feasible (OCR + page anchors); accuracy on scans/tables unproven.
Cross-document contradiction detection	Not stated by competitors; strongest wedge if error rates are material.	Hardest: false positives will kill trust. **Demo ≠ production regulatory product**; needs labelled test sets.
Regulatory change monitoring	Already offered by Rimsys, RegDesk, Qualio (Growth). Table stakes, not a wedge.	Feasible for CDSCO notices; thin value alone.

Legal/regulatory note: CDSCO FAQ states its replies are for public awareness, not professional/legal use — Evidentra must cite official text, not paraphrase it as authority.

11. Major risks and five assumptions to validate

Risks:

- Cause of delays is mostly inspection/QMS audit, not dossier quality
- Regulatory text drifts (draft 2026 timelines; Oct 2025 workflow change) → stale logic
- Wrong-answer liability; users over-trust output
- Consultants see tool as threat; CDSCO builds AI pre-screen itself
- Small, price-sensitive market (₹19 Cr base SAM)

Assumptions:

1. A1 Documentation gaps cause a material share of CDSCO queries
2. A2 Consultants will pay for dossier QA
3. A3 Tool finds real gaps/contradictions at useful precision
4. A4 ≥₹15k/pack or ≥₹3 L/yr is acceptable
5. A5 Firm/consultant counts support SAM

12. Validation experiments

Assumption	Cheapest test	Measurable pass criterion
A1 Gaps drive queries	Interview 15 RA leads/consultants; collect 10 anonymised CDSCO query letters and tag cause	≥50% of query items traceable to missing/inconsistent documents
A2 Demand	Landing page + paid pilot offer to 40 consultants; 5 pre-sold audits	≥3 paid pilots at ≥₹10k; ≥20% reply to outreach
A3 Accuracy	Manual-first: run 10 real dossiers (with consent) through a prototype; two experts label gaps	Precision ≥85%, recall ≥70% vs expert baseline; ≤1 critical false-negative per 10
A4 Willingness to pay	Van Westendorp + 3 price-point quotes in interviews	Median acceptable price ≥₹15k/pack or ≥₹3 L/yr
A5 Counts	Request licence list (RTI to CDSCO/State FDAs; SUGAM public data); scrape consultant directories	Verified licensees ≥1,500 Class B–D firms; ≥300 consultancies

13. What can be claimed today vs unproven

Supportable now:

- CDSCO requires structured, multi-part, online dossiers across risk classes (primary)
- A parliamentary committee reported repeated queries and opacity; govt drafted shorter timelines (secondary)
- Rival tools exist; few state India-specific support; Pure Global does

Unproven:

- That document errors cause CDSCO queries/delays; any % rework
- Time saved, accuracy, willingness to pay, consultant demand
- Number of licensees/consultants; market size beyond assumption
- That competitors lack a feature (only "not stated")

14. Final recommendation and key sources

REVISE: narrow to consultants + Class B/C dossiers; run A1–A3 within ~6 weeks; proceed to build only if A1 and A2 pass, stop if A1 <25% of query items or no paid pilots.

1. cdsco.gov.in/…/Acts-and-rules/Medical-Devices-Rules — MDR 2017 and notifications (P)
2. cdsco.gov.in/…/MDfAq24.pdf — CDSCO FAQ on MDR 2017, 2024 (P)
3. cdsco.gov.in/…/Medical-Device-Diagnostics — forms and SUGAM pathways (P)
4. medicaldialogues.in — parliamentary panel on CDSCO, 13 Mar 2025 (S)
5. pureglobal.com/services/pricing — closest public competitor pricing (S)